

SIR CHARLES GAIRDNER OSBORNE PARK
HEALTH CARE GROUP**CANNABIS WITHDRAWAL
MANAGEMENT
QUICK REFERENCE GUIDE**URN: Family name: Given name(s): Gender: DOB:

Signs of intoxication	Signs of withdrawal		
	Onset	Symptoms	Factors affecting severity
Euphoria Increased appetite Increased pulse Confusion Restlessness Hallucinations Delusions Anxiety / panic Paranoia	50–70% of dependent cannabis users will experience four or more withdrawal symptoms Commence on day 1 Peak at day 2–4 Symptoms usually last 2–3 weeks Occasional late development of anger and aggression up to two weeks after ceasing use	Anger / aggression Decreased appetite / weight loss Irritability Nervousness / anxiety Restlessness Sleep difficulties (including strange dreams) Cravings Sweating <i>Less common:</i> Depressed mood Paranoia	Psychiatric comorbidity Dose: amount, potency and preparation consumed History of aggression or violence Duration of current use and other past or current substance use history
Acute withdrawal treatment models			
General principles	Symptomatic management	Pharmacotherapy	Inpatient withdrawal
Most cannabis withdrawal can be managed in the community Symptomatic care is the mainstay for treatment Regular patient review is recommended Consider adding Nicotine Replacement Therapy for tobacco withdrawal (often mixed with cannabis)	Simple analgesia Antiemetics Antispasmodics Hydration Calm environment Psycho-education Decrease intake of caffeinated drinks	CONSIDER: Short course (1–5 days) of a benzodiazepine for anxiety Antipsychotic (e.g. Olanzapine 2.5–10 mg daily) for paranoid or psychotic features (Grade C)	Generally, not required. Patients may undergo withdrawal whilst admitted as an inpatient for other reasons. Acute withdrawal while an inpatient should be managed in the same way as low medical withdrawal
Cannabis hyperemesis			
Recurrent nausea, vomiting and cramping abdominal pain due to cannabis use.		Capsaicin cream 0.025–0.075% applied topically for symptoms of hyperemesis (Grade C)	
Post-withdrawal treatment options			
Consider an antidepressant for ongoing depressive symptoms (Grade D). Ongoing counselling GP / AMS and appropriate support people			